



P 3 9 · 2 4 P A G E S , P R I N T A B L E + F I L L A B L E

A VBAC Prep Planner

A twenty-four-page planner for the person considering a vaginal birth after cesarean, or being asked to consider a repeat cesarean. The planner does not advocate either way. It is the place to revisit the previous birth, prep the provider conversation, ask the hospital and practice policy questions, track the decision as it changes, and plan for whichever way the labor goes. With pages for the trauma, the partner, and the day-after.

Soothemade Notes

A S M A L L A P O T H E C A R Y O F P R I N T A B L E S · M A D E S L O W L Y

A P L A N N I N G A N D T R A C K I N G T O O L · N O T M E D I C A L A D V I C E

A note before you start

You are facing a decision that comes with a lot of noise.

Maybe you had a cesarean for your first birth, and this time you want a vaginal delivery if it is safe for you to attempt. Maybe you are not sure. Maybe your provider is recommending one direction, and you are not sure if you agree. Maybe you are getting opinions from people who have no business having opinions on your body.

This planner does not have an opinion either. It is not pro-VBAC. It is not pro-repeat-cesarean. The decision is between you and your provider, in conversation, over the course of this pregnancy. Some decisions get made early. Some get revisited late. Some get made during labor. All of those are legitimate.

What the planner does is hold the conversation. The questions you want to bring. The answers you get back. The decision as it is right now (which may not be the decision a month from now). The plans for whichever way it goes.

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This is a planning and tracking tool, not medical advice. The decision to attempt a vaginal birth after cesarean or to schedule a repeat cesarean is medical, situation-specific, and belongs to you and your obstetric provider. This planner does not recommend either direction. It records the conversation.

How to use this planner

- Read the front matter first, including the “we are not recommending” page.
- The trauma page is optional. Skip it if your first cesarean was uncomplicated and you do not need to revisit it.
- Bring the question pages to your appointments. Write the answers down in the planner, not on a sticky note.
- Update the decision log every few weeks. The decision can change. That is normal.
- Both back-half pages, “if labor starts” and “if it converts to cesarean,” are equal weight. The planner does not assume one outcome.

This planner is not making the decision

It is worth being explicit.

This planner does not tell you to attempt a VBAC. It does not tell you to schedule a repeat cesarean. It does not list percentages of success. It does not have a quiz that calculates your "VBAC score." It does not provide medical advice.

Why not.

Because the decision is situation-specific. The relevant factors include the reason for the previous cesarean, the type of uterine incision, the time since the previous birth, your current health, the baby's position, the hospital's policies, the provider's experience, and your own preferences. None of those things can be evaluated by a planner. They are evaluated by your provider, with you, in conversation.

What the planner can do:

- Help you prepare for that conversation
- Hold the answers you get
- Track your evolving stance
- Plan for both outcomes

What the planner refuses to do:

- Tell you what to choose
- Promise a specific outcome

- Quote statistics that may not apply to you

Where I am right now, leaning (you can be wrong, you can change):

- ☐ Strongly leaning VBAC
- ☐ Leaning VBAC, not certain
- ☐ Genuinely undecided
- ☐ Leaning repeat cesarean, not certain
- ☐ Strongly leaning repeat cesarean

The biggest factor in my current lean:

What might shift me:

The previous cesarean, revisiting

This page is optional. If your first birth was uncomplicated and you are not carrying anything heavy from it, skip to the next page.

For everyone else, the previous cesarean often comes back up during this pregnancy. The fear of doing it again. The grief that the first birth did not look like you hoped. The relief that you and the baby were safe. The anger at the person who said something wrong. Often all of those, in a single afternoon.

WHAT HAPPENED, IN MY OWN WORDS

This is for me, not for the provider. Write whatever needs to be said.
You can leave the lines blank if writing makes it worse.

THE HARDEST PART, NOW THAT I CAN SEE IT FROM HERE

WHAT I WISH HAD BEEN DIFFERENT

WHAT I WAS GLAD ABOUT

WHAT, IF ANYTHING, I AM SCARED OF REPEATING

A note on trauma. If your first birth was traumatic in a way you are still carrying, this planner cannot do the work of processing that. A perinatal therapist can. A trauma-informed doula can. Your partner sometimes can, but they may also be carrying it. The conversation with your current provider is also a place where the previous trauma deserves to come up.

You do not have to be “over” the first birth to make a decision about this one. You are allowed to bring all of it.

Finding or confirming your provider

Not every provider supports VBAC. Some practices flatly do not. Some support it conditionally. Some have a lot of experience. Some have none.

QUESTIONS TO ASK A PROVIDER OR PRACTICE EARLY

- ☐ Does your practice support VBAC attempts?
- ☐ What is the practice's general approach to TOLAC?
- ☐ At what hospital do you deliver, and what is that hospital's VBAC policy?
- ☐ How many VBAC attempts have you personally been involved in over the last year?
- ☐ Have you done VBAC attempts for someone with my specific previous cesarean situation?
- ☐ What would prompt you to recommend a repeat cesarean over a TOLAC for someone like me?
- ☐ What do you require of patients during labor for a TOLAC? (Continuous monitoring, IV placement, etc.)
- ☐ How do you handle the conversation if my labor stalls?

MY CURRENT PROVIDER

Name: _____

Practice: _____

Their position on a VBAC attempt for me, in their words:

Their delivery hospital: _____

DO I NEED A SECOND OPINION?

- ☐ Yes
- ☐ No
- ☐ Not sure yet

If yes, who I am considering:

Hospital + practice policy questions

Hospitals vary widely. Some have a labor-and-delivery setup that supports VBAC. Some require a 24-hour in-house OB. Some require specific staff coverage. Some flatly do not allow VBAC after a certain hour. Knowing the policy ahead of time prevents surprise.

HOSPITAL QUESTIONS

- ☐ Does this hospital allow TOLAC?
- ☐ Are there time-of-day restrictions on when TOLAC can be done?
(Some hospitals only allow it when an OB is in-house.)
- ☐ What monitoring is required during a TOLAC?
- ☐ How quickly can an emergency cesarean be performed if needed?
- ☐ Is anesthesia in-house 24/7?
- ☐ How many TOLACs do you do per month, roughly?
- ☐ What is the success rate at this hospital? (Take any number with context, but a hospital that does many TOLACs often has more experience.)

THIS HOSPITAL

Name: _____

Their TOLAC policy summary, in their words:

Any restrictions I should know about:

The TOLAC eligibility conversation

Your provider needs information from you and from your previous records. Many of these come up in the first or second prenatal visit.

WHAT MY PROVIDER WILL WANT TO KNOW (from records)

- ☐ The reason for the previous cesarean
- ☐ The type of uterine incision (this matters a lot, ask for the operative report if you do not have it)
- ☐ How many previous cesareans
- ☐ The interval between the previous cesarean and now
- ☐ Whether you have ever had a vaginal birth
- ☐ Any complications from the previous birth or surgery

WHAT I HAVE GATHERED

Reason for previous cesarean (in plain words):

Type of uterine incision (from operative report):

Time since previous cesarean to expected delivery:

Other relevant history I want to bring:

REQUEST FOR THE PREVIOUS OPERATIVE REPORT

Where it is held: _____

Whom I called: _____

Date requested: _____

Date received: _____

A note: get the operative report. It is the single most useful document. Your previous hospital can send it to your current provider. Do not assume your provider has it.

Decision-tracking log

The decision evolves. This page is a record of how it changes over the pregnancy.

ENTRY 1 DATE: _____

Current lean: _____

Why: _____

What happened this week that affected my thinking:

ENTRY 2 DATE: _____

Current lean: _____

Why: _____

What changed since last entry:

ENTRY 3 DATE: _____

Current lean: _____

Why: _____

What changed:

ENTRY 4 DATE: _____

Current lean: _____

Why: _____

What changed:

ENTRY 5 DATE: _____

Current lean: _____

Why: _____

What changed:

A note: it is okay to change your mind. It is okay to commit and then uncommit. It is okay to say "I am undecided" through the seventh month. The provider is not asking you to be certain. They are asking you to be honest about where you are.

Late-pregnancy timeline

What conversations to expect from the third trimester on.

AROUND 28 WEEKS

The provider often opens the VBAC conversation directly if it has not been opened yet. Bring your decision-tracking log. Bring your question list.

My 28-week appointment is: _____

Top question I want to ask:

AROUND 32-34 WEEKS

Many providers want a clearer direction by now. Hospital pre-registration sometimes asks about TOLAC vs scheduled repeat cesarean. This is where the planning gets more concrete.

My 32-34 week conversation went like this (notes after):

AROUND 36-37 WEEKS

For a scheduled repeat cesarean, this is often when it gets scheduled. For a TOLAC, this is often when the "what happens if you have not gone into labor by 40 weeks" conversation happens. Different providers have different policies on induction with a previous cesarean. Ask.

What my provider said about timing:

AROUND 39-41 WEEKS

If TOLAC is the plan and labor has not started, the provider may discuss the next steps. If a scheduled repeat is the plan, the date is set. Either way, the planner stays in the hospital bag.

What is currently on the calendar:

The “if labor starts” page

For the TOLAC plan, the day labor starts.

WHAT I WANT THE TEAM TO KNOW WHEN I ARRIVE

I am planning a TOLAC.

My provider's name: _____

Their phone (call them): _____

The reason for my previous cesarean was:

The incision type was: _____

Things that matter to me during the TOLAC, if they are safe:

- ☐ I prefer to walk during labor when monitoring allows
- ☐ I prefer intermittent monitoring when allowed
- ☐ I would like to try a vaginal birth as the first option
- ☐ My doula is: _____ (number: _____)
- ☐ My partner is staying with me throughout
- ☐ I want to know what the team is thinking, in plain language, at each decision point
- ☐ Other: _____

IF AT ANY POINT THE TEAM RECOMMENDS A CESAREAN

I want them to:

- ☐ Tell me clearly why
- ☐ Tell me how urgent it is
- ☐ Give me a moment to take a breath if there is time
- ☐ Talk to my partner so we are both informed

CALL LIST WHEN LABOR STARTS:

1. _____

2. _____

3. _____

The “if it converts to cesarean” page

A TOLAC that becomes a cesarean is not a failed TOLAC. It is a labor that ended a different way.

This page exists so that, if this happens, you walk into the cesarean with planning, not just resignation.

IF MY TOLAC CONVERTS TO A CESAREAN

WHAT I WANT IN THE OR

- ☐ Partner in the room (confirm hospital policy)
- ☐ My phone or camera with the partner for photos after
- ☐ Skin-to-skin with the baby in the OR, if possible
- ☐ Delayed cord clamping if it is safe
- ☐ The clear drape, if that is offered, so I can see them being born
- ☐ Quiet voices during the surgery
- ☐ Music if available (specific song or playlist:
_____)

WHAT I WANT TO KNOW IN PLAIN LANGUAGE

- Why the change is happening
- How urgent it is
- What to expect during the surgery

THE EMOTIONAL PART

You may need a day or a week or a year to process this. Use the P09 Postpartum Anxiety + PPD Journal if processing gets stuck.

AFTER

P06 C-Section Recovery Planner covers the next 5 weeks.

ONE LINE TO YOURSELF, IN ADVANCE:

The “if I schedule a repeat cesarean” page

Equal-weight planning. The repeat cesarean is a legitimate path. Many people choose it. Many providers recommend it. Some hospitals only allow it.

IF I AM SCHEDULING A REPEAT CESAREAN

THE SCHEDULED DATE: _____

WHAT I WANT IN THE OR

- ☐ Partner in the room
- ☐ The clear drape so I can see them being born, if offered
- ☐ Skin-to-skin with the baby in the OR, if possible
- ☐ Delayed cord clamping when safe
- ☐ Quiet voices during the surgery
- ☐ Music: _____
- ☐ Photos by the partner after delivery

THE NIGHT BEFORE

- ☐ Hospital bag packed
- ☐ Partner's bag packed
- ☐ Cutoff for eating, per the surgical team: _____
- ☐ The pediatrician is selected and noted in the chart
- ☐ Whom I am calling on the way to the hospital:

THE EMOTIONAL PART

A scheduled cesarean is not a "lesser" birth. It is a birth. It is okay to grieve the version that included labor if that was the version you wanted. It is also okay to feel relief. Both are common.

THE PRACTICAL PART AFTER

P06 C-Section Recovery Planner covers the next 5 weeks. It will be in the diaper bag.

Partner page

For the support person.

This is a different kind of birth planning than first-time pregnancies. The previous cesarean is a real event in your shared history. The decision now carries that weight.

THINGS THE SUPPORT PERSON CAN DO

DURING THE PREGNANCY

- ☐ Read the previous operative report with your partner
- ☐ Come to the provider conversations
- ☐ Listen to your partner's fears without trying to fix them
- ☐ Notice if your partner is carrying trauma from the first birth and gently suggest a therapist if so
- ☐ Help track the decision over time without pushing toward either outcome
- ☐ Do not have a strong personal opinion, or if you do, name it once and then let your partner lead

DURING LABOR (FOR A TOLAC)

- ☐ Bring the planner
- ☐ Be the person who asks "can you explain that in plain language" if something is unclear
- ☐ Watch your partner's energy, not just the monitors
- ☐ Be ready to advocate, if needed, for the things in the "if labor starts" page

IF IT CONVERTS

- ☐ Go with your partner to the OR
- ☐ Be the calm voice
- ☐ Take photos after the baby is here
- ☐ Be the person who fills in the rest of the planner because your partner will be recovering

IF IT IS A SCHEDULED REPEAT CESAREAN

- ☐ Drive your partner
- ☐ Be in the OR
- ☐ The post-op routine is in P06 C-Section Recovery Planner
- ☐ The first day home is mostly you doing the things your partner cannot do yet

THINGS TO SAY

- "Whatever we decide is fine with me."
- "I am with you, either way."
- "What do you want to ask the provider next time?"
- "How are you, separate from the decision?"

THINGS NOT TO SAY

- "Just do whatever the provider says."
- "I think you should ____."
- "Your friend's VBAC went great so you should ____."
- "It would be easier if we just ____."

A final letter

You are going to have a baby. The route they get here is one of two routes. Both are legitimate. Both are real.

Whichever path this pregnancy ends with, it is not a measure of you. The previous cesarean was not a failure. A second cesarean would not be a failure. A successful TOLAC would not be a victory over the first birth. They are all just births.

What the planner cannot do is decide for you. What it can do is hold the questions, the conversations, the plans, and the trauma if there is any. So that on the day, whichever day it is, you walk in with the work already done.

You are going to meet them. The how is between you and your provider.

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Soothemade *Notes*

*Made for the version of you no one tells you about, the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

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